

The one-afternoon underpayment check

Six steps to find out whether your insurers pay what your contracts say, using records you already have.

The six steps

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YOU NEED

Remittances for one insurer, 835 files or EOBs

AND

That insurer's fee schedule from your contract

TIME

An afternoon, once

Six steps

An underpaid claim doesn't look underpaid. The payment posts, the balance goes to zero, and the claim files itself under finished. The only way to know is to compare what the insurer allowed against what your contract says it owed.

1 Pick one insurer, and one product within it

Start with your highest-volume commercial insurer. Then narrow to one product or network, because a single insurer can pay different percentages by product: Florida Blue's own fee schedule says the percentage is "specific to each line of business." One insurer, one product, one fee schedule.

2 Pull 20 to 30 zero-balance claims for your five most-billed codes

Zero-balance on purpose: you are auditing the claims everyone considers finished. Take claims paid in the last three months, still inside every window on page 8. Leave out claims where this insurer was secondary; their allowed reflects what the primary paid.

3 Write down three numbers per line

What you billed, what the insurer **allowed**, and what your contract says the allowed should be. Allowed against contract is the whole test. Ignore paid: it moves with the patient's deductible and coinsurance. Two traps before you compare:

Match the place of service. Many contracts carry separate office and facility rates. The same code done in an ASC or hospital pays the surgeon less than in the office. Test a facility claim against the office rate and you'll find a shortfall that isn't there.

Use the year your contract names. If the contract pays a percentage of a published schedule, many fix a base year. Use that year's schedule for your locality, not the year of service, or you'll find gaps that aren't real and miss ones that are.

4 Check the code they paid

If the remittance pays a lower-valued code than you billed, the line was downcoded, even when the arithmetic on the paid code looks right. The signatures, and one bigger pattern for procedure practices, are on page 4.

5 Ignore scattered pennies. Look for repetition.

Rounding and one-off quirks are noise. What matters is the same code, the same insurer, the same gap, again and again. Rule out the lawful reductions on page 5 first.

6 Multiply by a year

Annual gap = average shortfall per short line × share of lines short × annual volume of that code, with that insurer, in that product

Where each number lives

The 835 is the electronic remittance advice (ASC X12 005010X221A1): how each insurer paid, reduced, or denied every line. Posting software reads what it needs to close a balance and ignores the rest, which is where underpayment sits. Paper EOBs and portal PDFs print the same facts in columns.

WHAT YOU NEED	835 SEGMENT AND FIELD	ON A PAPER EOB	WHY IT MATTERS
Your claim number	CLP01, patient control number	Patient account or claim number	Ties the line back to your system
Insurer's claim number	CLP07, payer claim control number	Claim or ICN number	Goes on every reconsideration
Claim status	CLP02	Status or "processed as"	1 to 3 processed, 2 as secondary, 4 denied, 22 reversal
Date of service	DTM*472 at the service line	Service dates	Needed for the deadline math
Place of service	On your claim, box 24B or the 837	Sometimes printed; often not	Decides office versus facility rate
Code paid	SVC01	Procedure code column	What the insurer adjudicated
Code you submitted	SVC06, present only when it differs from SVC01	Sometimes shown as "billed code"; often missing	If it differs from SVC01: downcode or recode. Posting usually ignores it
Billed charge	SVC02	Billed or charges	Confirms the line only
Allowed amount	AMT*B6, when the insurer sends it	Allowed or eligible	The insurer's version of your contract rate. The number to test
Paid amount	SVC03	Paid or plan payment	Moves with deductible and coinsurance
Adjustments	CAS: group code, reason code, amount	Adjustment or remark columns	CO yours, PR the patient's, OA other, PI insurer-initiated
Remark codes	LQ	Footnote codes	Often the only place the real reason appears
Provider-level adjustments	PLB	"Provider adjustments" at the end	Recoupments and offsets outside any claim line

No allowed amount on the line? Many insurers leave AMT*B6 out. Rebuild it: billed charge minus the contractual adjustments on that line, group CO and any PI. Never subtract PR. Deductible, coinsurance, and copay are part of the allowed amount.

What a downcode looks like

A downcode posts as a clean payment. The paid code carries its own correct rate, so nothing looks wrong unless you compare it to what you billed.

- **The paid code differs from the billed code.** 99215 paid as 99214, 99214 as 99213, 99205 as 99204. On the 835, SVC01 differs from SVC06.
- **The reason and remark codes.** The AMA's downcoding guidance lists CARC 150 ("payer deems the information submitted does not support this level of service"), CARC 186 ("level of care change adjustment"), and remark codes M85 and N610 as signs a claim was downcoded.
- **The allowed lands exactly on the lower code's rate.** If your contract pays 99214 at \$142 and 99213 at \$98, a \$98 allowed on a 99214 isn't rounding.

The bigger pattern in a procedure practice

An E/M billed with modifier 25 on the same day as a procedure, paid \$0 as included in the procedure, usually under CO-97. When the visit addressed a problem significant and separate from the procedure, and the note shows it, that \$0 is recoverable. When the visit was only the decision to do the procedure, it's usually correct. Sort your procedure days by E/M paid at zero; the count tells you which case you have.

The appeal for either kind is documentation: attach the note, and say whether the level rests on medical decision making or on total time on the date of service.

Where to get the files

- **Your clearinghouse.** Every 835 it delivered is downloadable, usually for at least a year. Ask for the raw files, not the formatted report.
- **Insurer portals.** Availity and the insurers' own portals let you pull remittances by check date, including insurers that bypass your clearinghouse.
- **Your billing company.** If they post for you, the files pass through them. They're your records; ask for the archive.

Keep at least twelve months. Florida's underpayment window on state-regulated plans runs twelve months from the payment date, and a file you don't have is a claim you can't make.

What isn't an underpayment

Most adjustments are correct. Disputing correct ones burns goodwill and buries your real claims in noise. Rule these out before you count anything.

- **Patient responsibility.** Deductible, coinsurance, and copay under group PR lower the paid amount, not the allowed. That's why step 3 tests allowed.
- **The routine contractual write-off.** When the allowed ties to the fee schedule, CO-45 is the deal you signed. When the allowed is below the schedule, the shortfall is hiding inside that same CO-45.
- **A legitimate multiple-procedure reduction.** Second and later procedures commonly pay at 50 percent, ranked by fee-schedule value, highest first. A reduction on your highest-valued procedure is a shortfall; the correct one isn't.
- **Bilateral procedures.** Modifier 50 is commonly priced at 150 percent of the single rate, not 200. Check your contract's bilateral rule. The real shortfall is the reverse: a bilateral priced at 150 and then cut again as a second procedure.
- **Assistant and co-surgeon lines.** Modifiers 80, 81, 82, AS, and 62 pay a set fraction of the surgeon's rate under most contracts. Test them against that fraction, not the full fee.
- **Professional and technical components.** A line with modifier 26 or TC pays only its component. Compare it to the component rate.
- **Visits inside a global period.** Follow-up visits paid \$0 inside a surgical global period, often CO-97 with remark M144 or N525, are usually correct. The exception is a visit for an unrelated problem, billed with the right modifier.
- **Code pairs that truly bundle.** Some pairs bundle under the national correct coding edits and can't be separated with a modifier (modifier indicator 0). Those \$0 lines are correct.
- **Secondary claims.** When this insurer paid second, allowed and paid reflect the primary's payment, often OA-23. That's why step 2 leaves them out.
- **Medicare sequestration.** Two percent of the Medicare payment amount, not of the allowed, under its own adjustment code.

One trap runs the other way. A deductible or coinsurance reason (CARC 1, 2, or 3) posted under group CO instead of PR is the insurer's coding error. As posted, it's your write-off and you can't bill the patient. Ask the insurer to correct the group code; those dollars are collectible from the patient.

A worked sample

Sample, synthetic figures. One commercial insurer, one product, an orthopedic practice, office claims only, 30 lines checked.

CODE BILLED	CODE PAID	PLACE	ALLOWED	CONTRACT	GAP	ADJUSTMENT	READ
99214	99214	Office	\$142.00	\$142.00	\$0.00	CO-45	Correct
99214	99213	Office	\$98.00	\$142.00	\$44.00	CO-150	Downcode, 9 of 30 lines
20610	20610	Office	\$88.40	\$96.10	\$7.70	CO-45	Rate shortfall, 12 of 12 lines
73030	73030	Office	\$31.20	\$31.20	\$0.00	CO-45	Correct
99203	99203	Office	\$121.00	\$121.00	\$0.00	PR-3	Copay, not a gap

Annualized. 99214 downcodes: $\$44.00 \times 30$ percent of lines $\times 1,400$ a year with this insurer = **\$18,480**.
20610 shortfall: $\$7.70 \times$ every line $\times 600$ a year = **\$4,620**. One insurer, two codes, **\$23,100 a year**.
Neither pattern is visible on a posted, zero-balance claim.

Reading the result

- **The downcode is a documentation dispute.** The reconsideration attaches the note and states the basis for the level.
- **The rate shortfall is arithmetic.** Contract page, recompute, done. If it hits every line of a code, the insurer probably loaded the wrong rate: see the contract-loading rule on page 9.
- **Zero gaps are results too.** A code that ties to contract on every line is one you can stop worrying about.

[illegible]

CODE	LINES CHECKED	LINES SHORT	AVG. GAP PER SHORT LINE	ANNUAL VOLUME, THIS INSURER	ANNUAL GAP

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What's still recoverable

Every finding has a clock. Two run at once: the insurer's own terms, and, for underpayments on state-regulated plans, Florida's statute. Work the soonest-expiring dollars first.

Insurer windows, contracted providers in Florida

INSURER	FIRST STEP	NEXT STEP
Florida Blue	Reconsideration within 1 year of the remittance date; required before appeal	Administrative appeal inside the same year
UnitedHealthcare	12 months from the EOB or remittance date	Reconsideration and appeal must both fit inside those 12 months
Aetna	Reconsideration within 180 days of the initial decision. Aetna's own state exceptions give Florida physicians 12 months on fully insured plans	Appeal within 60 days of the reconsideration decision; one level
Cigna	180 days from the initial payment or denial, "unless other timelines are required by state law"	One internal level, then arbitration per contract. Make the first one complete
Oscar	Florida: 365 days from receipt of the explanation of payment, unless your agreement sets another timeline	See the provider manual
Humana	About 120 days from the remittance; a working figure, confirm in your contract	60 days from the first-level decision
Medicare, traditional	Redetermination within 120 days of receiving the remittance	Reopening: 1 year for any reason, 4 years for good cause

Your contract can change these, and the deadline printed on the EOB or remittance controls over any table.

The Florida floor for underpayments

12 months from the payment date. A claim for underpayment by a physician, osteopath, chiropractor, podiatrist, or dentist must be submitted within 12 months after the insurer's payment. Section 627.6131(19), Florida Statutes; for HMOs, section 641.3155(17).

It can't be contracted away. The section may not be "waived, voided, or nullified by contract," section 627.6131(10). A shorter dispute clause doesn't erase the statutory window on covered business.

12 percent simple interest on overdue payments, section 627.6131(7); for HMOs, section 641.3155(6).

Where it doesn't apply. Self-funded employer plans under ERISA, even when a familiar Florida carrier administers them and the card looks the same. Medicare Advantage. Federal employee plans. Workers' comp, which runs on its own 45-day petition clock from receipt of the EOBR. Check funding status through the group number or benefits documents, claim by claim.

Triage rule. Underpayments on fully insured business stay alive 12 months everywhere. Denials run on the insurer clocks above: work the Humana ones first, and Aetna and Cigna self-funded ones next.

The one-page letter

Not a medical argument. A recompute: here is the contract rate, here is what you allowed, here is the difference, please reprocess.

[Practice letterhead] [Date] [Insurer], provider disputes, [address, fax, or portal reference from the provider manual]

Re: Request for reprocessing, underpayment against contracted rate

Tax ID [] · NPI [] · Claim [#] [or: [n] claims, list attached]

Claim [#], date of service [date], place of service [office or facility], CPT [code], was allowed at \$ []. The contracted rate under our agreement effective [date], [exhibit or section], is \$ []. *For a downcode, add:* the level billed is supported by [medical decision making, or total time of [] minutes]; the note is attached.

Please reprocess at the contracted rate and remit the difference of \$ [] [with 12 percent simple interest under section 627.6131(7), Florida Statutes]. Attached: remittance line; contract page, rate highlighted; recompute; proof of timely filing if needed. [Name, title, phone, email]

The interest clause belongs only on Florida fully insured plans; for an HMO cite section 641.3155(6).

File by pattern, the way each insurer takes it

One underpaid line isn't worth an hour. Every line from that insurer, that code, that quarter usually is. The instinct is the same everywhere; the mechanics aren't.

- **UnitedHealthcare.** 20 or more claims with the same administrative issue, no attachments needed, go in bulk online through Claims Research Project on the provider portal. Fewer go as single reconsiderations.
- **Cigna.** Its payment review form says fee-schedule or reimbursement issues for multiple patients "do not require individual appeals." Call customer service, or your Cigna contract representative if you're contracted, and log the reference number. A call doesn't pause the filing window.
- **Florida Blue.** One claim per reconsideration form. Send the pattern as a set of forms with the same explanation and contract page.
- **Aetna.** One member per appeal form; that member's dates of service can go together.
- **Everyone else.** Read the dispute section of the provider manual for batch rules before you send a list.

Three more rules

1. **Reconsideration first.** Faster, usually handled by claims staff, and it keeps the formal appeal in reserve.
2. **Use the channel the manual names for payment disputes.** A portal timestamps receipt. A dispute sent to the medical appeals queue can sit for months.
3. **If they say your rate is different,** you've found a contract-loading problem. Fixing it in their system stops the shortfall on every future claim, which is often worth more than the appeal.

If the afternoon isn't available, Clearia's diagnostic runs this check on every insurer and every line of the last twelve months, at no cost. We're paid only from what we recover. cleariagroup.com/the-diagnostic

General information, not legal advice. Statutes, rules, and insurer policies change; verify the current text and your own contracts before acting on any deadline or rate cited here. Figures on page 6 are synthetic.